VERIDIAN INSTITUTE OF MEDICAL SCIENCES

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SYNTHETIC SAMPLE - fictional patient, hospital and clinicians. Not a medical record. For software testing only.

FOLLOW-UP RADIOLOGY REPORT — CT THORAX (HRCT) & CHEST X-RAY PA

Patient Name: Mrs. SUNITA KAPOOR	**MRN / PID:** VIMS-RD-7731902	**Accession No:** ACC-20261203-0891
Age / Gender: 54 Y / Female	**Referring Clinician:** Dr. R. Menon (Pulmonology)	**Exam Date / Time:** 03-Dec-2026 / 11:20 AM
Modality: DX (X-Ray), Low-Dose CT Thorax	**Contrast:** Non-contrast volumetric	**Prior Exam Date:** 22-Oct-2026 (6-Week Interval)

CLINICAL INDICATION & COMPARISON

CLINICAL INDICATION: 54-year-old female with known Type 2 Diabetes completed a 14-day antimicrobial course for right lower lobe community-acquired lobar pneumonia and small parapneumonic effusion. Patient is currently afebrile, asymptomatic with complete resolution of cough, dyspnoea, and pleuritic pain. SpO2 99% on ambient air.
COMPARISON: Compared with prior volumetric HRCT Thorax and PA chest radiograph dated 22-Oct-2026 (ACC-20261022-0457).

FINDINGS & INTERVAL EVOLUTION

Lungs & Airways: Complete resolution of previously visualised dense lobar consolidation and prominent air bronchograms in the posterior and lateral basal segments of the right lower lobe. Centrilobular tree-in-bud nodules and adjacent ground-glass opacities in the right middle lobe have cleared entirely. No residual scarring, cavitation, or architectural distortion. Left lung remains clear.

Pleura: The previously noted 14 mm right-sided parapneumonic pleural effusion has resolved completely. Costophrenic angles are acute and clear bilaterally. No pleural thickening or loculation.

Mediastinum & Lymph Nodes: Previously reactive right hilar and subcarinal lymph nodes (largest was 12 x 9 mm) have regressed to normal sub-centimetre dimensions. Cardiac size and great vessels are normal.

Chest Radiograph PA: Confirms fully expanded lung fields, clear right costophrenic sulcus, and normal cardiothoracic ratio (CTR 0.44).

RADIOLOGICAL IMPRESSION (VISIT-OVER-VISIT EVOLUTION)

1. COMPLETE RESOLUTION of right lower lobe lobar pneumonia and associated 14 mm parapneumonic effusion compared to prior study of 22-Oct-2026.
2. Regression of reactive lymphadenopathy; clear parenchyma and normal pleural spaces bilaterally.
3. No further radiological surveillance required in the absence of recurrent clinical symptoms.

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